Personal History:

1- **Name:** For Identification & To Be Familiar with Patient.
2- **Age (Date of birth):** Certain Diseases Are Related to Certain Age Groups.
3- **Sex:** Some Diseases Are More Common in Males / Females.
4- **Blood group**
5- **Nationality:** Certain Diseases Are Related to Certain Countries.
6- **Home & Residence (better to the place of birth):** Certain Diseases Are Related to Certain Areas.
7- **Source of information:** (mother – father – grandmother)
8- **Date of admission & Time.**
9- **Source of referral.**
10- **Religion of parents. (accordingly)**

Note: The Personal History should be mentioned in one paragraph.

e.g.: male patient named AHMED EMAD SAMI, Muslim 5 yrs. in old, O+ve born in TUZ and living KIRKUK of Iraq. The mother is the primary historian and is deemed reliable. He is admitted at 15th of july,2017 at 7:00 pm by referring from DR… the history taken at(date) and the examination done at(date)

Chief compliant and duration:

1- The main symptoms that necessities medical consultation.
2- max 3 words)
3- Duration of the conditions
4- Only in the patient own words (don't use medical terms)
5- Sort Them in Chronological Manner. (the first is first etc...)

Diarrhea ……frequent bowel motion
Convulsion…… abnormal involuntary movements
Constipation …… Infrequent or difficult evacuation of feces
Jaundice …… Yellowish discoloration of skin and sclera
Edema ……swelling of various parts of the body
Vomiting …… forceful expulsion of gastric contents
Hematemesis …… blood in vomitus
Hemoptysis …… blood in sputum
Dyspnea …… difficulty of breathing
Cyanosis …… bluish discoloration of skin and mucus membrane
Polydipsia …… excessive thirst and water drinking
Polyphagia …… Excessive hunger and eating
Pain in the right hypochondrium …… pain in the upper right quadrant of the abdomen
Palpitation …… awareness of heart beats
Delayed of motor development …… Delayed walking setting etc…
Anorexia…… loss of appetite
Xerostomia…… dry mouth
Dysgeusia …… altered taste sensation
Dysphagia …… difficulty swallowing
Odynophagia …… …. pain on swallowing
Flatulence …… Belching
Dyspepsia …… Indigestion
Nausea …… …… Feeling sick

e.g. of cc and duration: difficulty of breathing for 3 days **before** admission to hospital

History of Presenting Illness (HOPI):

A- The Patient Is Known Case of ……. (Chronic Disease).

B- The Patient Was Well till / On Usual Status till …… (Hours - Days - Weeks - Months) Back When He/She Started to Has ……. (Complaint).

C- Analysis of Complaint:

I) General Symptoms:

1- **Fever:** Onset, Course, Duration, Pattern, Grade, Associated with (Rigors - Convulsions - Skin Rash).
2- **Wight Changes:** Onset, Course, Duration, How Much? Associated Symptoms.
3- **Fatigue / Malaise:** Onset, Course, Duration, Grade, Associated Symptoms.
4-**sleep / mood /energy**

II) GIT Symptoms:

1- Dysphagia / Odynophagia: Onset, Course, Duration, Type (Food - Fluid), Level, Associated Symptoms.
2- Regurgitation: Onset, Course, Duration, Associated Symptoms.
3- Loss of Appetite: Onset, Course, Duration, Associated Symptoms.
4- Nausea: Onset, Course, Duration, Associated Symptoms.
5- Vomiting: Onset, Course, Duration, Content, Color, Odor, Relation to Eating, Precipitated by Nausea, Associated Symptoms.
6- Abdominal Pain: Site, Onset, Course, Duration, Character (Nature), Radiated / Referred / Shifted, Aggravating & Relieving Factors, Severity (Measured by: Interfering with Daily Activities, 0-10 Score), Associated Symptoms.
7- Abdominal Swelling: Site, Onset, Course, Duration, Associated Symptoms.
8- Diarrhea: Onset, Course, Duration, Content, Color, Odor, Frequency, Consistency, Associated Symptoms.
9- Constipation: Onset, Course, Duration, Type (Partial - Complete), Presence of Anal Pain, Associated Symptoms.
10- Jaundice: Onset, Course, Duration, Who Notice? Stool Color, Urine Color, RUQ Pain.
11- Bleeding : Onset , Course , Duration , Content , Color , Odor .

III) Cardio-Pulmonary Symptoms:

1- Cough: Onset, Course, Duration, Character, Frequency, Aggravating & Relieving Factors, Sputum, Relation to Position, Associated Symptoms.
2- Sputum: Onset, Course, Duration, Color, Odor, Amount, Relation to Position, Consistency.
3- SOB: Onset, Course, Duration, Type, Severity, Grade, Associated Symptoms.
4- Chest Pain: Site, Onset, Course, Duration, Character, Radiation, Aggravating & Relieving Factors, Associated Symptoms (Abdominal Pain, Nausea, Vomiting, Sweating, Fever).
5- Palpitation: Onset, Course, Duration, Rhythm, Occurrence (Exposure - Exertion), Frequency, Associated Symptoms.
6- Syncope: Onset, Course, Duration, Occurrence (Exposure - Exertion), Frequency, Associated Symptoms.
7- Wheeze: Onset, Course, Duration, Occurrence (Exposure - Exertion), Associated Symptoms.
8- Cyanosis: Site (Oral - Extremities), Onset, Course, Duration, Occurrence (Rest - Exertion), Relation To Position, Associated Symptoms.
9- Lower Limb Swelling: Onset, Course, Duration, Site (Uni/Bilateral), Ascending / Descending, Result From Trauma? Associated Symptoms.

IV) CNS Symptoms:

1- Symptoms Suggestive of Higher Functions Involvement: Onset, Course, Duration, Type, Associated Symptoms.
2- Symptoms Suggestive of Cranial Nerves (CNs) Involvement: Onset, Course, Duration, Type,Function Impairment (Sensory, Motor, Gustatory), Associated Symptoms.
3- Weakness: Site, Onset, Course, Duration, Grade, Features of Increased Intra-Cranial Pressure (ICP), Associated Symptoms.
4- Abnormal Movement: Site, Onset, Course, Duration, Type, Features of Increased ICP, Sphincter Function, Associated Symptoms.
5- Headache: Site, Onset, Course, Duration, Grade, Character, Related to Trauma, Associated Symptoms.

Notes:
- **Symptoms of Higher Function:** Loss of Consciousness - Memory - Speech, … Etc.
- **Symptoms of CNs Involvement:** Anosmia, Visual Field Defect, Diplopia, Abnormal Sensation in The Face, Symptoms of Facial Nerve Palsy, Decrease Hearing / Vertigo, Dysphagia, Nasal Regurge, Tongue Deviation
- **Symptoms of Increased ICP:** Headache, Blurred Vision, Projectile Vomiting.

V) Genito-Urinary Symptoms:

1- Pain: Site, Onset, Course, Duration, Character, Radiation, Aggravating & Relieving Factors, Severity, Associated Symptoms.
2- Polyuria: Onset, Course, Duration, Frequency, Volume, Associated Symptoms.
3- Dysuria: Onset, Course, Duration, Associated Symptoms.
4- Hesitancy / Urgency / Urine Retention: Onset, Course, Duration, Associated Symptoms.
5- Incontinence: Onset, Course, Duration, Related to Neurological Problem? Associated Symptoms.
6- Discharge: Onset, Course, Duration, Color, Amount, Odor, Consistency, Itching.

VI) Musculo-Skeletal Symptoms:

1- Bone / Joint / Muscle Pain: Site, Onset, Course, Duration, Character (sharp stabbing / dull ache / burning*?*), Radiation, Aggravating & Relieving Factors, Severity, Frequency (Morning / Night), Any Swelling, Limitation of Movement, Trauma, Stiffness, Associated Symptoms.
2- Joint / Muscle Swelling : Site , Onset , Course , Duration , Trauma , Stiffness , Associated Symptoms .

VII) Skin Symptoms:

1- Skin Rash: Onset, Course, Duration, Distribution, Type, Associated Symptoms.
2- Itching: Onset, Course, Duration, Associated Symptoms.
3- Hyper/Hypo Pigmentation: Onset, Course, Duration, Associated Symptoms.
4- Hirsutism / Alopecia: Onset, Course, Duration, Associated Symptoms.
5- Ulcer: Onset, Course, Duration, Itching, Loss of Sensation, Associated Symptoms.

D- Review The Symptoms of the Affected System:

-Example: In The Case of Abdominal Pain, You Have to Review Other GIT Symptoms Which Include: (Dysphagia, Heartburn, Jaundice, Hematemesis, Constipation, Diarrhea, Melena, Bleeding Per Rectum.)

E- Systemic Review:

- **CNS:** Headache, Dizziness, Change in Behavior, Loss of Consciousness, Weakness, Abnormal Movement.
- **GIT:** Dysphagia, Heartburn, Jaundice, Hematemesis, Constipation, Diarrhea, Melena, Bleeding Per Rectum.
- **Cardio-Pulmonary:** Cough, Hemoptysis, Dyspnea, Chest Pain, Palpitations, Syncope, Claudication.
- **Urogenital:** Loin Pain, Dysuria, Polyuria, Hematuria, Urethral Discharge.
- **Skin & Musculoskeletal:** Pain, Muscle Wasting, Pigmentation, Ulcers.
- **Hematology:** Easy Fatigability, Petichiae, Gum Bleeding, Pallor.

F- Hospital Course:

- **Investigations.**
- **Medications & Interventions.**
- **Improving OR Not.**

Notes:
Onset – when did the symptom start? / was the onset acute or gradual?
Duration – minutes / hours / days / weeks / months / years
Severity – e.g. if symptom is shortness of breath – are they able to talk in full sentences?
Course – is the symptom worsening, improving, or continuing to fluctuate?
Intermittent or continuous*?* – is the symptom always present or does it come and go?
Precipitating factors – are there any obvious triggers for the symptom?
Relieving factors – does anything appear to improve the symptoms e.g. an inhaler
Associated features – are there other symptoms that appear associated e.g. fever / malaise
Previous episodes – has the patient experienced these symptoms previously?

G- Any new event that may occur during hospitalization ((worsening of baby condition

ask about sleep + activity (smile/fatigue) + feeding all will have decreased)

H-Outcome (the condition of patient at the moment of taking history)

NOTE: convert these points to story and write it.

Example HOPI:
This is a 2-month old infant who presented to the After Hours Clinic for fever and fussiness. The mother states that the patient would normally take 2oz of Similac formula every 2-3 hours. He would also get up in the middle of the night for feedings every 3 hours. However, over the last 24 hours, he has not been taking his feedings as frequently.He has taken 3 feedings of 2oz over the last 24 hours. He did not wake during the night for any feeding. The mother also noted that the patient was increasingly fussy during the day. He would cry when left along for any amount of time, which was a deviation from his normal routine. He was able to be consoled when held however. He has had 6 wet diapers in the last 24-hours, and he normally has 6-8 per day. He typically has stool in the diaper 3 times per day, with 3 normal stools noted in the last 24 hours.
The mother took the patient's tympanic membrane temperature which was found to be 103F. The mother became worried about the infant's high temperature because he has an underlying heart condition, as well as a history of UTI, so decided to take him into the
clinic. There, the patient was noted to be febrile and somewhat lethargic, so they recommended him for admission to the hospital.
The mother states that the patient has not had any cough, diarrhea, vomiting, rash, sick contacts, or wheezing.

Prenatal, natal and post-natal

Pre-natal (Ante-natal)

1. ANC (does the mother attend regular visits to private doctor or PHC)
2. Disease of mother during pregnancy (infections like Toxoplasma-HIV-Rubella-
3. Cytomegalovirus-Chickenpox-Hepatitis --- TORCH infection appear as fever + skin rash
4. + joint swelling --- other diseases like D.M and hypertension and anemia)
5. Drugs taken during pregnancy
6. Exposure to radiation during pregnancy
7. Smoking
8. Bleeding
9. Previous abortion or neonatal death
10. Admission to hospital for any reason
11. Vaccine
12. Number of child
13. history of Infertility

Natal

1. Place of delivery (at home or hospital)
2. Type of delivery (vaginal or caesarian section CS) if CS what is the indication? Elective or emergency?
3. Duration of delivery (normal or prolonged)
4. Any complication during labor
5. Instrument use in the delivery
6. Gestational age (pre-term or term or post-date or post-term)
7. Weight of the baby (normal AGA, Small SGA, Large LGA)
8. Hx of birth trauma, respiratory difficulty ,convulsions .

Post-natal (first 28 days of life)

1. Immediate crying
2. Time of discharge from the hospital (24 hours – 48 – more)
3. NICU admission
4. When the baby passed urine (within first 24 hours) and meconium (the first stool) (within first 48 hours)
5. Movement
6. Neonatal jaundice, Cyanosis (fetal distress), Fit, bleeding
7. Baby developed other diseases? Treatment?

e.g. Patient was born at term to a G4P3 mother via spontaneous vaginal delivery. The pregnancy was complicated by chronic hypertension and diabetes. The mother was treated with methyldopa and labetalol during pregnancy. She did not require insulin. The patient weighed 3.29kg (30th %) at birth and was 50cm long (50th %) with head circumference of 36cm (50th %)……etc. **complete other findings**

Past medical history:

1. Previous Similar Conditions.
2. Previous Hospital Admissions.
3. Previous Surgical Interventions.
4. Previous Blood Transfusion.
5. Chronic diseases
6. Previous infections (measles, others)

Past-surgical:

1. Previous operations (indication, type, outcome)
2. Circumcision in male and ear piercing in female (at which age, any complications)
3. Hospitalization

Drug history:

1. Drug taking by mother (chronic use)
2. Drug taking by baby (chronic use)
3. Allergy to drug and other substances

Feeding history:

1- Breast feeding

- Way of feeding (using both right and left breast each feeding time) Regular (at least every 3 hours) or on demand
- Any problem with feeding (large nipple, others)

2- Bottle feeding

- Way of feeding
- Way of preparation
- Type of formula use (lactose free, soy milk formula, others)
- Way of sterilization of the bottle (boiling, Washing, brushing)
- Number of bottles
- Number of feeding
- Regular (at least every 3 hours) or on demand
- Any problem occurs after bottle feeding (diarrhea, others)
- Put the bottle in freeze for cooling.

3- Mixed feeding (breast and bottle feeding)

4- Semi-solid or solid food :at which age start solid food? Type of it?

- At which time given
- Type
- Any problem occurs after this feeding

5- Weaning: at which age milk was taken off his diet

6- Pica: Ask if the child eat soil, wood or other things (caused by iron deficiency anemia – Ca deficiency – lead poising)

Immunization history:

1- Take the vaccines on regular Iraqi schedule or not?
2- On national immunization day only?
3- Type of vaccine? At which age?
4- Any complications (fever, pain, convulsion, rash, excessive crying) pertussis vaccine cause convulsion shock
5- Time of last vaccine

Notes: RV not give to baby after 3 and 8 months of age / BCG not give to baby after 1 year of age / BCG should return if the scar not appear.

Age	VACCINE
24 hours after birth	BCG, HBV , OPV
2 months	DTP-HepB-Hib, OPV, RV
4 months	DTP-Hib, OPV, RV
6 months	DTP-HepB-Hib, OPV, RV
9 months	Measles, Vit.A 100,000 U
15 months	MMR
18 months	DTP-Hib, OPV, Vit.A 200,000 U
4-6 years	DTP, MMR, OPV

Name	In Arabic	Type	Route
BCG	بي سي جي	Live attenuated TB bacteria	ID
HBV	إلتهاب الكبد الفيروسي ب	Inactivated hepatitis B virus	IM
OPV	شلل الأطفال فموي	Live attenuated Polio virus	Oral
RV	الفايروس العجلي	Live attenuated Rotavirus	Oral
Measles	الحصبة المنفردة	Live attenuated measles virus	SC
MMR	الحصبة المختلطة	Live attenuated, mumps, rubella viruses	SC
DTP	اللقاح الثلاثي	Diphtheria and Tetanus Toxoid + inactivated pertussis bacteria	IM
DTP-Hib	اللقاح الرباعي	DTP + Inactivated Influenza virus	IM
DTP-HepB-Hib	اللقاح الخماسي	DTP + Inactivated Influenza virus + Inactivated hepatitis B virus	IM

Developmental:

1- Gross motor (sit, stand, walk)
2- Fine motor (Grasp, move object from hand to hand, etc.)
3- Social (smile, laugh, etc.)
4- speech
5- vision
6- hearing

Motor development

❑ *During infancy*
- At birth: turns head from side to side
- 3m: supports head
- 5m: sits with trunk support, palmar grasp
- 6m: sits with pelvic support
- 7m: sits without support
- 9m: crawling, pincer grasp
- 10m: standing
- 12m: walks with support
- 13-18m: walks without support

❑ *During early childhood*
- 1.5y: ascends stairs in childhood manner
- 2y: descends stairs in childhood manner
- 3y: can pedal tricycle – ascends stairs in adult manner
- 4y: descends stairs in adult manner

❑ *During late childhood*
- At 6y: can dress him (or her) self


6 months


7 months


9 mon


12 months

Mental development

❑ *During infancy*
- 1m: follows light & fixes objects
- 2m: social smile
- 3m: listens to music
- 4m: laughs
- 6m: recognizes parents & prefers mother
- 8-9m: says monosyllables (mama-dada)
- 12m: waves 'bye-bye'

❑ *During early childhood*
- 1y: says 2-3 words
- 1.5y: says about 10 words
- 2.5y: says telegraphic sentences
- 3y: gives full name, age & sex
- 4y: counts up to 10

Family history:

1- health of parents, brothers and sisters
2- family history of same illness
3- family history of chronic disease or infectious disease
4- family history of unexplained death
5- order of baby in the family
6- age of the baby before or after him
7- any illness in the family (as Inherited / Genetic Diseases.)
8- Consanguinity. Rh ABO incompatibility.
9- History of Sudden Death, Cancers.

Social history:

1- rural or urban
2- source of water
3- income
4- level of education and occupation of parents
5- crowding index (number of persons/number of rooms) below 3 is normal and above 3 is crowding
6- any domestic animals
7- ventilation
8- smoking
9- sewage disposal
10- History of Recent Travelling (When, Where, How Many Times).
11- - Special Habits (Alcohol Abuse, Smoking, Drug Abuse, Sexual Activities, …).

* Note: For Smoking, You Have to Ask About: **Duration + Type + Smoking Index** (Pack-Year).

Obstetrical and gynecological history:(only for girls below 18)

- Age of Menarche, Regularity, Character (Heavy OR Not), Duration, Volume.
- Age of Menopause.
- Use of Contraception Methods.
- Duration of Delivery, Mode of Delivery of Each One & Complicated OR Not.

